Adjuvant trastuzumab emtansine (T-DM1)

T-DM1 by IV infusion every 21 days for 14 postoperative cycles, unless recurrence or unacceptable toxicity requires a change.

Draft — independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Low platelets / bleeding

T-DM1

Keep platelet checks. New bruising or bleeding needs advice; heavy or persistent bleeding needs urgent help.

3 Heart pumping function

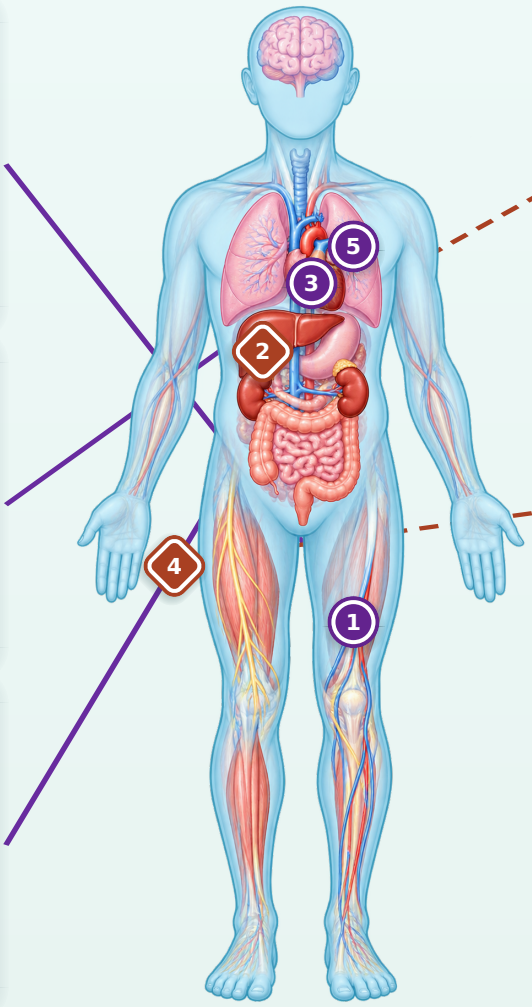
T-DM1

Keep echoes. Report new breathlessness, swelling or reduced exercise tolerance.

5 Lung inflammation

T-DM1 / radiotherapy context

New cough or breathlessness needs assessment, particularly if radiotherapy overlaps.



2 Liver injury

T-DM1

Liver tests are checked before each cycle. Report yellow skin / eyes or dark urine.

4 Peripheral nerves

T-DM1

Report numbness, tingling or difficulty with daily tasks before the next dose.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Trastuzumab emtansine (T-DM1)

- Low platelets and bleeding
- Liver injury, rarely serious
- Numbness or tingling
- Heart pumping-function reduction
- Rare serious lung inflammation
- Fatigue, nausea and infusion reactions

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Support for this phase

T-DM1 is low emetogenic risk. Confirm the prescribed anti-sickness plan and infusion-reaction instructions; check the full drug name.

Bloods and cardiac checks

Prior-treatment checks → Baseline FBC, liver and renal tests → Before every cycle: platelets, liver and renal tests. Every 12 weeks; sooner if abnormal / symptoms.

Visits and practical support

Ask about travel, work, fertility, menopause, exercise and help at home. The team arranges and reviews clinical checks; the patient should not have to calculate the monitoring timetable.

Follow-up and bone health

If an aromatase inhibitor or ovarian suppression is prescribed, assess fracture risk and baseline DEXA as indicated. Repeat timing depends on the result, risk and bone treatment; it is not an automatic scan for everyone.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

CUH · MON–FRI 8AM–5PM

087 181 9645

Acute Oncology Service

CUH · WEEKENDS / BANK HOLIDAYS 9AM–5PM

021 4922000

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00659/

Sources and review status are available online. Follow your own prescription and hospital instructions.